



SurgPath Final
Temporary Copy

Case: [REDACTED]
 Collected: [REDACTED]
 Ordered by: [REDACTED]

Page 1 of 3

Patient: [REDACTED]
 ID: [REDACTED]

FINAL DIAGNOSIS

- A) Thyroid, right lobectomy: Papillary thyroid carcinoma, forming 2.5 cm tumor mass, confined within the thyroid capsule. Focally suspicious for lymphovascular invasion. One parathyroid lymph node involved by metastatic papillary carcinoma (See synoptic report).
- B) Thyroid, left lobectomy: Papillary thyroid carcinoma, forming a 0.8 cm tumor mass, confined within the thyroid capsule. Two additional microscopic foci (each <1.0 mm) of papillary thyroid carcinoma also seen in random parenchymal sections. Hyperplastic nodules. Lymphovascular invasion is present. (See synoptic report).
- C) Lymph nodes, central compartment, excision: Five of eight lymph nodes involved by metastatic papillary thyroid carcinoma. Two unremarkable parathyroid glands. Benign thymic tissue is also present.

A: Thyroid Gland
HISTOLOGIC TYPE:
 Papillary carcinoma
 Classical (usual)
TUMOR FOCALITY:
 Multifocal (specify):
 Bilateral

ICD-O-3

Carcinoma, papillary, thyroid
 8260/3

Site: thyroid, NOS
 C73.9

TUMOR SIZE:
 Greatest Dimension: 2.5 cm

6-7-12
 RD

PRIMARY TUMOR (pT):
 pT2: Tumor more than 2 cm, but not more than 4 cm, limited to thyroid

REGIONAL LYMPH NODES (pN):
 pN1a: Nodal metastases to Level VI (pretracheal, paratracheal and prelaryngeal/Delphian) lymph nodes

NUMBER OF LYMPH NODE(S) INVOLVED: 6

NUMBER OF LYMPH NODE(S) EXAMINED: 9
LYMPH NODE, EXTRANODAL EXTENSION:
 Not identified

DISTANT METASTASIS (pM):
Not applicable

MARGINS:
Margins uninvolved by carcinoma

LYMPH-VASCULAR INVASION:
Present:
Focal (less than 4 vessels)

EXTRATHYROIDAL EXTENSION:
Not identified

ADDITIONAL PATHOLOGIC FINDINGS:
Adenomatoid nodule(s) or Nodular follicular disease (eg, nodular hyperplasia, goitrous thyroid)
Parathyroid gland(s):
Within normal limits

SPECIMEN RECEIVED

- A) RIGHT THYROID LOBE AND ISTHMUS, F.S.
- B) LEFT THYROID LOBE
- C) CENTRAL COMPARTMENT LYMPH NODES

FROZEN SECTION

- A) Right thyroid lobe and isthmus: Papillary thyroid carcinoma. Seen with Dr.

GROSS DESCRIPTION

A) Received fresh labeled "right thyroid lobe and isthmus" is a beefy red thyroid lobe measuring 5.0 x 4.0 x 2.5 cm and weighing 13 grams. The external surface is inked black and the specimen is serially sectioned to reveal a 2.5 x 2.0 x 1.6 cm circular tan/brown, partially cystic lesion with a friable cut surface. Focal areas of fibrosis are seen. The lesion appears contained within the thyroid capsule and occupies approximately 80% of the lobe. A portion of the mass is submitted for the Touch preparations and a representative frozen section

of the lesion are submitted in A1. Additional permanent section of lesion A2 through A5. Normal thyroid A6.

B) Received in formalin labeled "left thyroid lobe" is a lobe of thyroid weighing 7 g, and measuring 4.0 x 3.0 x 2.0 cm. The red/tan capsule surface is focally disrupted. A small amount of fatty soft tissue is adherent to the specimen. The specimen is inked on its surgical margins with black ink, serially sectioned, and reveals a firm white/tan lesion measuring 0.8 cm in greatest dimension. This lesion focally abuts the inked capsule. Additional smaller nodules measuring up to 0.7 cm in greatest dimension are also identified. The remainder of the thyroid parenchyma is beefy red/tan. The specimen is entirely submitted as follows: B1 entirely submitted indurated lesion, B2 through B6 remainder of thyroid.

C) Received in formalin labeled "central compartment lymph nodes" are four irregular portions of yellow lobulated node-bearing adipose tissue measuring 3.0 x 2.0 x 1.0 cm. Dissection yields multiple possible lymph nodes measuring up to 0.4 cm in greatest dimension. The specimen is entirely submitted in C1 through C4.

CLINICAL INFORMATION
PAPILLARY THYROID CANCER

Criteria	Yes	No
Diagnosis Discrepancy		/
Primary Tumor Site Discrepancy		/
HPAA Discrepancy		/
Prior Malignancy History		/
Dual/Synchronous Primary Noted		/
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	Date Reviewed: 5/24/12	

5/24/12